

## ULCM M1 — LLM Data-Extraction Prompt

**Version:** 1.0 **Purpose:** Extract structured data from one included paper (systematic review or primary study) for the ULCM M1 Rapid Review. **Companion doc:** ULCM\_M1\_Extraction\_Fields\_by\_RQ.docx (canonical field list). **Protocol reference:** ULCM Rapid Review Protocol v9 July (Appendix D — schema; Appendix F.4 — this prompt's protocol version; Appendix F.5 — span validator).

---

### How to run

1. Per record, run this prompt **k = 3** times with the three paraphrase variants (\$ Paraphrase variants below). Majority vote at field granularity. Any per-field disagreement → add to `fields_flagged_for_human`.
2. Model config (per protocol Appendix F.1):
  - Primary extractor: latest Claude flagship. temperature = 0.0, top\_p = 1.0, seed = 42, max\_tokens = 4096.
  - Verbatim-span validator (a separate call, Appendix F.5): Claude mid-tier. temperature = 0.0, max\_tokens = 512.
3. Every non-null field's span is then run through the span-validator prompt. Any verdict: "fail" → human re-extraction of that field.
4. **All quantitative fields used in synthesis are 100% human-verified** against the source PDF before use in any synthesis output (protocol §4.4.1).
5. Store every run — prompt version, model API string, raw response, timing — as JSONL under `/llm_logs/` for audit.

**Inputs to substitute per run:** - `{{record_id}}` — EPPI Study ID. - `{{unit_of_extraction}}` — "review" or "primary\_study". - `{{rq_tags_hint}}` — the RQ(s) the record was screened into (from full-text screening). - `{{segmented_full_text}}` — the paper's full text pre-segmented into labelled sections (Abstract, Methods, Participants, Intervention, Comparator, Outcomes, Results, Discussion, Cost if separate, plus Table 1, Table 2, ... for each results table).

---

### SYSTEM

You are a structured data extractor for a rapid systematic review on ultra-low-cost, group-based psychological interventions for adult depression in low- and middle-income countries (LMICs). The review is commissioned by 3ie for StrongMinds and follows the protocol referenced above.

Your job is to extract one paper into a single JSON object matching the schema below. You do three things and only three things:

1. Extract every field you can from the document.
2. For every non-null value, attach a verbatim span ( $\leq 40$  words) copied character-for-character from the source and record the section it was drawn from (e.g., "Methods 2.1", "Table 3", "Results, para 4").
3. Return JSON only. No prose before or after.

You NEVER:

- Speculate, infer, paraphrase, or fabricate a value the document does not explicitly state.
- Round numbers, convert currencies, adjust for inflation, or re-scale effect sizes. Extract the digits as they appear.
- Invent a new controlled-vocabulary value. If none of the closed-list values fits, set the field to "other" and populate "other\_specify" with a  $\leq 10$ -word verbatim-anchored description.
- Draw a supporting span from the abstract when the field is a numeric result reported in the methods, results, or tables. Prefer the primary results table where one exists.

If a field cannot be populated from the document, set its value to null and its span to "". Add the field ID to fields\_flagged\_for\_human.

If two passages contradict each other, take the value from the primary results table and add the contradicting passage to fields\_flagged\_for\_human.

Return one JSON object matching the schema below. Populate every field using the shape { "value": ..., "span": "", "section": "" } — there are no bare values in your response.

---

## USER

UNIT OF EXTRACTION: { {unit\_of\_extraction} } # "review" | "primary\_study"  
RECORD TIER: { {record\_tier} } # "full" | "reduced" — only when  
unit\_of\_extraction = "primary\_study"  
SOURCING REVIEW IDs: { {source\_review\_ids} } # array of EPPI sids; only when  
unit\_of\_extraction = "primary\_study"  
RQ TAGS FROM SCREENING (hint, not binding): { {rq\_tags\_hint} }

Extract the paper below into the schema at the end of this message.

---

A. PICOST recap (for orientation only — inclusion has already been decided by full-text screening; do NOT re-litigate inclusion here)

---

- Population: Adults ( $\geq 18$  y) with depression / mixed anxiety-depression / CMD, any severity. Perinatal women meeting depression criteria are eligible. Mixed adult/adolescent samples are eligible only if the adult subgroup is separately reported.

- Intervention: Brief, structured, group-based psychological interventions delivered by non-specialists or lay providers. For the dose, SSI, and stepped-care questions (RQ7, RQ8, RQ9, RQ12, RQ14), specialist-delivered evidence is eligible (flag it).
- Comparator: Any (TAU, waitlist, attention control, active, none).
- Outcomes: Depression symptoms on a validated instrument; remission; response; anxiety; functioning; attendance/adherence/fidelity; acceptability; cost; durability at  $\geq 3$  months.
- Study designs: Entry-point reviews are SR/MA/umbrella/NMA/IPD-MA/meta-regression/component-MA. Primary studies (reached through in-scope reviews) are RCT-individual, RCT-cluster, or quasi-experimental-controlled. Single-arm and pre-post-only designs are ineligible.
- RQ11 carve-out: Non-case / universal-prevention samples are eligible IF the intervention is in scope AND at least one depression-relevant outcome is reported for the non-case sub-sample. Set `rq11_only = true`.

---

## B. Extraction rules

---

1. Every value's span quotes the source character-for-character ( $\leq 40$  words). Numeric spans must contain the digits exactly as they appear (no rounding, no unit conversion).
2. For cost figures: extract in original currency and year. Do NOT convert to USD, PPP, or a base year. The downstream pipeline handles CPI inflation + market FX.
- 2a. For effect-size precision: extract SE as reported by the source. NEVER back-calculate SE from a CI, p-value, or t-statistic in the extraction step. If the source reports only a CI, extract the CI bounds and leave `eff_se = null`. Downstream pipeline decides whether to derive SE.
- 2b. For every time-point at which an effect is reported (post, 3m, 6m, 12m), extract the arm-level analytic Ns (`n_at_*_int`, `n_at_*_ctrl`). These are the Ns actually contributing to the estimate at that time-point, not the enrolled Ns.
3. Provider perspective is the headline cost figure. Societal-perspective figures are recorded if given but never as the headline.
4. For controlled-vocabulary fields, use only values from the closed lists in Section D of this prompt. If none fits, set to "other" and populate "other\_specify" with  $\leq 10$  words + a span.
5. Do NOT extract a primary ingredient. Multi-select every technique the source describes into `int_techniques`. The primary ingredient is assigned later at synthesis using the  $\geq 50\%$  planned-session-minutes rule.
6. If a field's `extractor_confidence < 0.7`, add the field ID to `fields_flagged_for_human`.
7. If the source is a systematic review reporting only pooled estimates:
  - `design.value` = "Systematic-review" or "Meta-analysis";
  - `eff_value` = pooled estimate; `pop_n` = total N across included studies; `dose_band` = "Not-reported" unless the review reports a

modal dose.

8. Attempt every field. If a field is inapplicable to this unit (e.g., rob\_fatal\_flaw on a review record), set value = null and span = ""; do NOT add it to fields\_flagged\_for\_human.
9. rq\_tags: multi-select every RQ this record contributes to (use the screening hint as a starting point, but add any RQ you find evidence for and remove any that the record does not actually address).
10. template\_section: multi-select from the mapping table in Section E. A record can feed multiple sections.

=====

[REVISION — pending user review]

Rules for primary-study records inside the capped RQ set

=====

11. Capped RQ set (protocol §4.1.2): primary-study extraction is performed only for RQ5, RQ6, RQ7, RQ8, RQ9, RQ10, RQ12, RQ13, RQ14, RQ16. Other RQs are answered from review-level records only.
12. Two record tiers (see §3A.3 of the extraction fields document):
  - record\_tier = "full": the study reports at least one comparable SMD (or Cohen d / mean-diff on a named depression instrument), OR the intervention dose in D.5 fields, OR a per-participant cost figure. Extract every field.
  - record\_tier = "reduced": in-scope primary study inside the sourcing review but not driving the effect / dose / cost tabulations. Extract only: sid, cit, doi, yr, design, country, geo\_focus, pop\_age, pop\_n, sev\_strat, int\_name, int\_techniques, dose\_band, dose\_n\_sessions, fac\_type, del\_format, eff\_direction, eff\_value (headline only), amstar2\_band (inherited), rob\_fatal\_flaw, source\_review\_ids, rq\_tags, workstream, template\_section. Set every other field to value = null and span = "" WITHOUT adding to fields\_flagged\_for\_human.
13. Sourcing-review link: for every primary-study record, populate source\_review\_ids with the sid(s) of the review(s) the study was reached through. If more than one, list all.
14. Deduplication (protocol §4.1.2): a primary study cited by more than one review is extracted ONCE. If this record has already been extracted (indicated by RECORD TIER = "dedup\_append"), do NOT re-extract the schema; instead, return an object with only sid and source\_review\_ids populated, listing the sids of all sourcing reviews (existing + new).
15. Linked publications: where the same primary study has multiple publications (protocol, main results, cost, long-term follow-up), the extractor is prompted with the segmented text of the INDEX

publication (most recent / most complete). Any cost or long-term follow-up data from a companion publication has been merged into the segmented text ahead of extraction and tagged with its section label (e.g., "Cost 4.1 [companion: doi/XXXX]"). Populate the fields from those companion sections normally.

16. RCT quality items feed the fatal-flaw ROB judgement (rob\_fatal\_flaw).

For each of rct\_randomisation\_method, rct\_allocation\_concealment, rct\_blinding\_participants, rct\_blinding\_outcome\_assessor, rct\_itt\_analysis: assign a verdict from {Y, N, Unclear, NA} plus a verbatim span. Blinding of participants is usually infeasible in psychological trials — code as NA where the source acknowledges this rather than Unclear.

=====  
=====  
[REVISION — pending user review — Fatal-Flaw ROB checklist]  
=====  
=====

17. Fatal-Flaw ROB (FF-ROB) checklist (protocol §4.4; template supplied by the review team). For every primary study, run the 5-criterion checklist below. Each sub-item takes a single-select verdict of "No — proceed" or "Yes — STOP, high ROB". A "Yes" on ANY sub-item is a fatal flaw and sets ffrob\_overall\_decision = "High ROB — fatal flaw"; all sub-items "No" → ffrob\_overall\_decision = "Assign for full ROB assessment".

Criteria (verbatim from the checklist):

C1. Severe confounding or baseline non-equivalence

C1a. Treatment and comparison groups differ substantially at baseline on key outcome determinants, and authors do not adequately adjust.

C1b. Selection into treatment is clearly related to expected outcomes (e.g., self-selection reflecting motivation), and no credible strategy to address.

C2. Differential attrition likely to bias results

C2a. Overall attrition exceeds ~30% AND differs meaningfully between groups (or > 30% with no info on differential). If attrition is not discussed at all, do NOT downgrade.

C2b. Attrition is likely related to outcomes AND authors do not demonstrate robustness (baseline comparisons of completers vs drop-outs, tipping-point / bounds analysis, etc.).

C3. Outcome measurement clearly influenced by treatment status

C3a. Outcome assessors knew treatment status AND no safeguards were implemented.

C3b. Primary outcomes are self-reported AND subject to bias (social desirability, recall). NOTE for depression trials: PHQ-9 and equivalents are self-report by design. Fail C3b

only when self-report is coupled with unblinded rating conditions OR the source itself flags the bias risk.

C4. Evidence of selective reporting

C4. Outcomes listed in the methods section are omitted from the results reporting. Base the verdict ONLY on methods vs results within the paper; do NOT consult the protocol or trial registry at this stage.

C5. Serious contamination or intervention deviations

C5a. Large proportion of comparison group received the intervention, or large spillover, AND flagged as an issue by the author(s). Latent, unstated contamination → No.

C5b. Authors state that intervention implementation differed fundamentally from intended.

For every sub-item populated with "Yes", populate the matching \_span field with a verbatim quote and the \_notes field for that criterion with a ≤ 40-word rationale. Populate criterion-level notes ONLY when the study fails on that criterion.

18. Propagation to rob\_fatal\_flaw (§3.11 of the extraction fields doc):

- ffrob\_overall\_decision = "High ROB — fatal flaw"
  - rob\_fatal\_flaw = "Fatal-flaw-present"
  - also populate ffrob\_failed\_criteria with the failing sub-items
- ffrob\_overall\_decision = "Assign for full ROB assessment"
  - rob\_fatal\_flaw = "No-fatal-flaw"
- Review-level record → rob\_fatal\_flaw = "Not-applicable"; all ffrob\_\* fields null.

19. Downstream consequence of a fatal flaw:

- The record is EXCLUDED from effect-size, dose, and cost tabulations.
- The record is retained at record\_tier = "reduced" for descriptive / narrative purposes.
- Extractor does not gate inclusion on FF-ROB; that gating happens at synthesis. Populate every field as usual; the FF-ROB verdict is a flag, not a stop-word.

=====  
=====  
[REVISION — pending user review — AMSTAR-2 itemised checklist]  
=====  
=====

20. AMSTAR-2 (Shea et al. 2017) is the sole quality appraisal for every review-level record (systematic review, meta-analysis, meta-regression, IPD-MA, network-MA, umbrella review, dismantling-component MA). AMSTAR-2 is NOT applied to primary studies (they use the FF-ROB checklist, rules 17–19).

21. Protocol mode: RAPID CRITICAL-DOMAINS. Seven critical items are

mandatory on every review:

I2 Protocol registered before review, deviations justified  
I4 Comprehensive literature search  
I7 Excluded-studies list with justification  
I9 Satisfactory RoB technique for individual studies  
I11 Appropriate meta-analytic methods (if MA performed)  
I13 RoB accounted for in interpreting results  
I15 Publication bias assessed (if quantitative synthesis)  
Nine non-critical items (I1, I3, I5, I6, I8, I10, I12, I14, I16)  
are optional in rapid mode but should be coded on borderline  
reviews or where a critical item's verdict depends on them.

22. Per-item response coding:

- I1, I3, I5, I6, I10, I12, I13, I14, I15, I16: Yes / No  
(I11, I12, I15 also permit "No MA conducted")
- I2, I4, I7, I8: Yes / Partial Yes / No
- I9: Yes / Partial Yes / No / Includes only NRSI / Includes only RCTs

23. For every item populated, extract a verbatim span ( $\leq 40$  words) from the review's methods, participants, results, or discussion — never from the abstract. For critical items span + notes are mandatory. For non-critical items span is required whenever the verdict is not a clear Yes.

24. Confidence-band derivation (rapid mode):

Let CRITWEAK = count of critical weaknesses among I2, I4, I7, I9, I11, I13, I15. A "critical weakness" is a verdict of "No" OR (for I2, I4, I7 only) "Partial Yes". "No MA conducted" on I11 or I15 does NOT count as a weakness. "Includes only RCTs" / "Includes only NRSI" on I9 does NOT count as a weakness.

Let NONCRIT = count of non-critical items with verdict "No" or "Partial Yes" (when non-critical items are coded).

Derived band (populate amstar2\_band\_derived):

- "High" if CRITWEAK = 0 AND NONCRIT  $\leq$  1
- "Moderate" if CRITWEAK = 0 AND NONCRIT > 1
- "Low" if CRITWEAK = 1
- "Critically-low" if CRITWEAK > 1

Populate amstar2\_band with the derived band unless a stated override applies (inherited from an in-corpus umbrella review) — in which case set amstar2\_band = umbrella's rating, set amstar2\_inherited = true, populate amstar2\_inherited\_source with the umbrella's sid, and record the override reason in amstar2\_override\_reason.

25. Review-only RQs (RQ1, RQ2, RQ3, RQ4, RQ11, RQ15, RQ17, RQ18):

- Primary-study drill-down is not performed for these RQs. Every

finding fed into Section 6 of the report is inherited from a review-level record.

- The amstar2\_band of the sourcing review IS the confidence rating for the RQ finding at synthesis.
- Extractor does not gate inclusion on amstar2\_band, but must correctly compute it — the synthesis step uses it to assign Section 2 dashboard letters (H/M/L/Ins.) and to route Critically-low findings into §10 Evidence gaps.

26. For a review already appraised by AMSTAR-2 inside a corpus umbrella review (e.g., meta-analyses inside Jeitani et al. 2024): do NOT re-rate. Set amstar2\_inherited = true, populate amstar2\_inherited\_source, and copy the umbrella's band into amstar2\_band. Populate individual amstar2\_i{N} fields with null and empty spans — do NOT flag them for human review.

=====  
=====  
[REVISION — pending user review — sub-modules, multi-outcome, RQ matrix]  
=====  
=====

27. Sub-module decomposition (§3.4A of the extraction fields doc). When the source paper or review reports at the sub-module level for a named programme (IPT-G role-disputes / role-transitions / grief / interpersonal-deficits; PM+ managing-stress / managing-problems / get-going / social-support / staying-well; HAP behavioural-activation-core / problem-solving / relapse-prevention; THP, CBT-brief, PST, MI, SSI, Friendship Bench, etc.), populate the int\_submodules field with values from the vocabulary keyed to the record's int\_name / programme\_family. If the source only reports programme-level content without sub-module granularity, leave every §3.4A field null and DO NOT flag for human review.

Sub-module vocabulary (keyed by programme):

IPT-G / IPT-classic:

Interpersonal-inventory-assessment / Grief / Role-disputes /  
Role-transitions / Interpersonal-deficits /  
Termination-relapse-prevention

PM+ (Problem Management Plus):

Managing-stress / Managing-problems / Get-going-keep-doing (BA) /  
Strengthening-social-support / Staying-well-looking-forward

HAP (Healthy Activity Program):

Engagement-values / Behavioural-activation-core /  
Activity-monitoring-scheduling / Problem-solving-component /  
Relapse-prevention



THP (Thinking Healthy Programme, perinatal):  
Preparing-for-baby / Baby-arrival / Mother-and-baby /  
Baby-and-others / Health-information-module

Friendship Bench:  
Kufungisisa-framing / Kusimudzira / Kusimbisa /  
Peer-support-groups

CBT-brief / Common-Elements-CBT:  
Psychoeducation-CBT / Behavioural-activation /  
Cognitive-restructuring-core / Problem-solving-CBT /  
Relaxation-stress-management / Exposure /  
Relapse-prevention-CBT

PST / PST-based:  
Problem-orientation / Define-problem / Generate-solutions /  
Evaluate-select / Implement-review

BA-based (stand-alone):  
Activity-monitoring / Activity-scheduling / Values-clarification-BA /  
Skills-training-BA / Relapse-prevention-BA

MI-based:  
Engaging / Focusing / Evoking / Planning

SSI-protocolised:  
Psychoeducation-SSI / Values-clarification-SSI /  
Behavioural-commitment-SSI

Peer-support programmes:  
Structured-peer-listening / Skill-modelling /  
Mutual-aid-group-process / Referral-facilitation

Other-programme:  
Other-submodule (specify)

submodule\_session\_share\_pct is a JSON object mapping each populated sub-module to its % of planned session minutes; the values must sum to  $\leq 100$ . submodule\_contribution captures the source's stated contribution of each sub-module (e.g., "role-transitions carried the largest SMD (0.62); role-disputes null"). submodule\_evidence\_type marks the design (Dismantling-MA / Component-MA / Network-MA / Meta-regression / Head-to-head-trial-sub-arm / Narrative / Session-share-only / Other).

28. Additional DEPRESSION outcomes — repeating structure (§3.8A). §3.7 captures ONE primary depression outcome; §3.8 captures follow-up SMDs at fixed post / 3m / 6m / 12m. §3.8A adds an array named

additional\_outcomes where every entry describes ONE {depression construct × instrument × time-point × subgroup × adjustment} combination the study reports.

SCOPE — DEPRESSION OUTCOMES ONLY. §3.8A entries are restricted to depression constructs measured on validated depression instruments.

Non-depression outcomes are captured elsewhere in the schema:

- Cost outcomes → §3.7 cost\_\* fields.
- Engagement (attendance, completion, adherence, fidelity, acceptability) → §3.9 engage\_\* fields.
- Safety (adverse events, suicidality, referral pathway) → §3.9 ae\_\*, safety\_\* fields.
- Psychometric outcomes (RQ18) → §3.10 psychom\_\* fields.
- Attrition → §3A.4.3 attrition\_\* fields.

If the study reports an anxiety, functioning, wellbeing, or QoL outcome alongside depression, do NOT create an additional\_outcomes entry for it. Note it in the top-level notes field.

Extract every reported depression outcome — including studies that report the same depression construct on multiple instruments (a study reporting BOTH PHQ-9 and BDI-II gets two entries per time-point), subgroup breakdowns (severe stratum, perinatal, women, urban sites), and adjusted / unadjusted estimates.

Each entry uses the shape defined in the schema below with: outcome\_id, outcome\_construct, outcome\_scale, outcome\_scale\_direction, outcome\_is\_primary, outcome\_timepoint\_weeks, outcome\_timepoint\_label, outcome\_timepoint\_verbatim, outcome\_eff\_metric, outcome\_eff\_value, outcome\_eff\_se, outcome\_eff\_ci\_lo, outcome\_eff\_ci\_hi, outcome\_eff\_p, outcome\_n\_int, outcome\_n\_ctrl, outcome\_arm\_int\_mean, outcome\_arm\_int\_sd, outcome\_arm\_ctrl\_mean, outcome\_arm\_ctrl\_sd, outcome\_eff\_direction, outcome\_adjusted\_flag, outcome\_adjustment\_covariates, outcome\_subgroup, outcome\_source\_location, outcome\_quote.

Rules:

- No cap on the number of entries per study — extract every reported outcome × time-point.
- The primary outcome entries at post-intervention should be consistent with the §3.7 fields (they are a superset, not a replacement).
- outcome\_timepoint\_verbatim quotes the source's own label ("week 12", "T3", "end of therapy") so downstream code can normalise to outcome\_timepoint\_weeks.

29. Study-to-RQ contribution matrix (§3.12A). Every extracted record also populates an array named rq\_contributions with ONE entry per RQ the study contributes to. A study contributing to RQ5, RQ7, and RQ14 has three entries in rq\_contributions.

Each entry uses:

- `rq_id` RQ1 ... RQ18
- `rq_contribution_type` Primary-evidence / Supporting-evidence / Contextual-evidence / Contradictory-evidence / Descriptive-only / Not-applicable
- `rq_contribution_summary`  $\leq$  30-word summary of what the study says about this RQ
- `rq_contribution_strength` High / Moderate / Low / Insufficient (derived from `amstar2_band` for review records; from `ffrob_overall_decision` + `record_tier` for primary studies)
- `rq_contribution_direction` Confirms / Refutes / Qualifies / Neutral / Not-applicable
- `rq_contribution_data_fields` List of field IDs (e.g., ["`eff_value`", "`eff_3m_smd`", "`dose_band`"])
- `rq_contribution_template_section` Duplicates `template_section` at the per-RQ granularity
- `rq_contribution_quote` Verbatim span backing the summary

#### Rules:

- The LLM re-reads the paper against the full RQ set at extraction time; it may ADD RQs not in `rq_tags_hint` and REMOVE RQs the hint suggested but the paper does not actually address.
- Every `rq_id` in `rq_contributions` must also appear in the top-level `rq_tags` field (§3.12).
- Fatal-flaw studies still get a `rq_contributions` entry per RQ, but `rq_contribution_strength` is set to "Insufficient" and `rq_contribution_type` to "Descriptive-only".
- The exported extraction sheet materialises `rq_contributions` as long-format: one row per {`sid`  $\times$  `rq_id`}. The synthesis team filters by `rq_id` to build per-RQ evidence tables and Section 2 dashboard confidence letters.

---

### C. JSON output schema (uniform provenance shape)

---

Every field uses { "`value`": ..., "`span`": "", "`section`": "" } except `extractor_confidence` (float 0–1) and `fields_flagged_for_human` (array of strings). Return exactly this schema; do not add fields; do not omit fields.

```
{
  // Study identification (D.1)
  "sid":      { "value": "{ {record_id} }", "span": "", "section": "" },
  "cit":      { "value": "<Vancouver citation>", "span": "", "section": "" },
  "doi":      { "value": "<DOI/URL or null>", "span": "", "section": "" },
  "yr":       { "value": "<int|null>", "span": "", "section": "" },
  "lang":     { "value": "<D.8 lang>", "span": "", "section": "" },
  "doctype":  { "value": "<D.1 doctype>", "span": "", "section": "" },
```

```

// Design and context (D.2)
"design":      { "value": "<D.8.1>", "span": "", "section": "" },
"country":    { "value": ["<ISO-3>"], "span": "", "section": "" },
"stream":     { "value": "Stream1-Barbui-anchored" | "Stream2-seed" |
"Internal", "span": "", "section": "" },
"geo_focus":  { "value": "SSA" | "other-LMIC" | "HIC-UMIC" | "mixed", "span":
"", "section": "" },
"setting":    { "value": ["<D.2 setting list>"], "span": "", "section": "" },
"pop_age":    { "value": "Adult (≥18)" | "Perinatal women" | "Mixed
adult+adolescent" | "Other", "span": "", "section": "" },
"pop_n":      { "value": <int|null>, "span": "", "section": "" },
"pop_descr":  { "value": "<free text>", "span": "", "section": "" },

// Severity stratum (D.3)
"sev_strat":  { "value": "<D.8.2>", "span": "", "section": "" },
"sev_scale":  { "value": "<D.3 sev_scale>", "span": "", "section": "" },
"sev_cutoff": { "value": <num|null>, "span": "", "section": "" },
"sev_quote":  { "value": "<verbatim>", "span": "", "section": "" },

// Intervention (D.4)
"int_name":   { "value": "<name>", "span": "", "section": "" },
"int_techniques": { "value": ["<≥1 of D.8.3a>"], "span": "", "section": "" },
"int_adaptations": { "value": ["<0+ of D.8.3b>"], "span": "", "section": "" },
"int_theory":  { "value": "<free text or null>", "span": "", "section": "" },
"int_quote":   { "value": "<verbatim ingredient description>", "span": "",
"section": "" },

// Sub-module decomposition (§3.4A) [REVISION]
"int_submodules": { "value": ["<from vocabulary keyed by
programme>"], "span": "", "section": "" },
"submodule_session_share_pct": { "value": { "<sub-module>": <int 0-100> },
"span": "", "section": "" },
"submodule_contribution": { "value": "<free text per sub-module>", "span": "",
"section": "" },
"submodule_evidence_type": { "value": "Dismantling-MA" | "Component-MA" |
"Network-MA" | "Meta-regression" | "Head-to-head-trial-sub-arm" | "Narrative" |
"Session-share-only" | "Other", "span": "", "section": "" },
"submodule_quote": { "value": "<verbatim if any submodule_* set>",
"span": "", "section": "" },
"submodule_manual_reference": { "value": "<free text>", "span": "", "section":
"" },

// Dose (D.5)
"dose_n_sessions": { "value": <int|null>, "span": "", "section": "" },
"dose_session_min": { "value": <num|null>, "span": "", "section": "" },
"dose_total_contact_min": { "value": <num|null>, "span": "", "section": "" },
"dose_homework_min": { "value": <num|null>, "span": "", "section": "" },
"dose_band": { "value": "<D.8.4>", "span": "", "section": "" },
"dose_freq": { "value": "Daily" | "Weekly" | "Fortnightly" | "Monthly" |
"Self-paced" | "Irregular" | "Not-reported", "span": "", "section": "" },

```

```

"dose_duration_wk":      { "value": <num|null>, "span": "", "section": "" },
"dose_delivered_pct":    { "value": <num|null>, "span": "", "section": "" },
"dose_quote":           { "value": "<verbatim>", "span": "", "section": "" },

// Facilitator and delivery (D.6)
"fac_type":             { "value": "<D.8.5>", "span": "", "section": "" },
"fac_train_h":          { "value": <num|null>, "span": "", "section": "" },
"fac_super":            { "value": "<D.6 super>", "span": "", "section": "" },
"fac_clinical_background": { "value": "<see notes>", "span": "", "section": "" },
"del_format":           { "value": "<D.8.6>", "span": "", "section": "" },
"del_modality":         { "value": "In-person" | "Phone" | "Video" | "Hybrid", "span": "",
"section": "" },
"del_lang":             { "value": ["<languages>"], "span": "", "section": "" },
"group_size_mean":       { "value": <num|null>, "span": "", "section": "" },
"group_size_range":     { "value": "<min-max or null>", "span": "", "section": "" },

// Outcomes and effects (D.7)
"out_primary":          { "value": "<free text>", "span": "", "section": "" },
"out_scale":            { "value": "<D.7 scale>", "span": "", "section": "" },
"out_timepoint":        { "value": <num|null>, "span": "", "section": "" },
"eff_metric":           { "value": "SMD" | "Cohen d" | "Mean-diff" | "RR" | "OR" |
"Remission-rate" | "Other", "span": "", "section": "" },
"eff_value":            { "value": <num|null>, "span": "", "section": "" },
"eff_se":               { "value": <num|null>, "span": "", "section": "" },
"eff_ci_lo":            { "value": <num|null>, "span": "", "section": "" },
"eff_ci_hi":            { "value": <num|null>, "span": "", "section": "" },
"eff_n_int":            { "value": <int|null>, "span": "", "section": "" },
"eff_n_ctrl":           { "value": <int|null>, "span": "", "section": "" },
"eff_direction":        { "value": "Favours-intervention" | "Null" | "Favours-control" |
"Unclear", "span": "", "section": "" },
"eff_quote":            { "value": "<verbatim>", "span": "", "section": "" },

// Durability / temporal effects (needed for RQ8, RQ14). For every time-point:
// extract the estimate, its SE, its CI, and the arm-level Ns behind it.
// Extract SE as reported; NEVER back-calculate SE from CI or p-value.
"eff_post_smd":         { "value": <num|null>, "span": "", "section": "" },
"eff_post_se":          { "value": <num|null>, "span": "", "section": "" },
"eff_post_ci_lo":       { "value": <num|null>, "span": "", "section": "" },
"eff_post_ci_hi":       { "value": <num|null>, "span": "", "section": "" },
"n_at_post_int":        { "value": <int|null>, "span": "", "section": "" },
"n_at_post_ctrl":       { "value": <int|null>, "span": "", "section": "" },
"eff_3m_smd":           { "value": <num|null>, "span": "", "section": "" },
"eff_3m_se":            { "value": <num|null>, "span": "", "section": "" },
"eff_3m_ci_lo":         { "value": <num|null>, "span": "", "section": "" },
"eff_3m_ci_hi":         { "value": <num|null>, "span": "", "section": "" },
"n_at_3m_int":          { "value": <int|null>, "span": "", "section": "" },
"n_at_3m_ctrl":         { "value": <int|null>, "span": "", "section": "" },
"eff_6m_smd":           { "value": <num|null>, "span": "", "section": "" },
"eff_6m_se":            { "value": <num|null>, "span": "", "section": "" },
"eff_6m_ci_lo":         { "value": <num|null>, "span": "", "section": "" },

```

```

"eff_6m_ci_hi":    { "value": <num|null>, "span": "", "section": "" },
"n_at_6m_int":    { "value": <int|null>, "span": "", "section": "" },
"n_at_6m_ctrl":   { "value": <int|null>, "span": "", "section": "" },
"eff_12m_smd":    { "value": <num|null>, "span": "", "section": "" },
"eff_12m_se":     { "value": <num|null>, "span": "", "section": "" },
"eff_12m_ci_lo":  { "value": <num|null>, "span": "", "section": "" },
"eff_12m_ci_hi":  { "value": <num|null>, "span": "", "section": "" },
"n_at_12m_int":   { "value": <int|null>, "span": "", "section": "" },
"n_at_12m_ctrl":  { "value": <int|null>, "span": "", "section": "" },
"remission_pct_int": { "value": <num|null>, "span": "", "section": "" },
"remission_pct_ctrl": { "value": <num|null>, "span": "", "section": "" },
"temporal_quote": { "value": "<verbatim, if any eff_*m_smd set>", "span": "",
"section": "" },

```

```

// Additional DEPRESSION outcomes — repeating array (§3.8A) [REVISION]
// SCOPE: depression outcomes only. Non-depression constructs are captured
// in dedicated blocks (cost §3.7 cost_*, engagement §3.9 engage_*,
// safety §3.9 ae_/safety_*, psychometrics §3.10 psychom_*, attrition
// §3A.4.3). Do NOT create an entry for anxiety/functioning/wellbeing/QoL.
//
// Extract EVERY depression outcome × instrument × timepoint × subgroup ×
// adjustment combination. No cap. The primary-outcome-at-post entries
// here should be consistent with §3.7 (superset, not replacement).
"additional_outcomes": [
  {
    "outcome_id":          { "value": "<auto: sid-out-NN>", "span": "", "section":
"" },
    "outcome_construct":   { "value": "Depression-symptoms" | "Depression-
remission" | "Depression-response" | "Depression-recovery" | "Depression-relapse",
"span": "", "section": "" },
    "outcome_scale":       { "value": "PHQ-9" | "PHQ-2" | "CES-D" | "CES-D-10" |
"HAMD" | "HDRS" | "MADRS" | "BDI" | "BDI-II" | "EPDS" | "QIDS" | "Zung-SDS" |
"GDS" | "DASS-21-depression-subscale" | "SCL-90-depression-subscale" | "SRQ-20-
depression-items" | "K10-depression-cutoff" | "MINI-depression-module" | "CIDI-
depression-module" | "SCID-depression-module" | "Other-depression-instrument",
"span": "", "section": "" },
    "outcome_scale_direction": { "value": "Higher-is-worse" | "Higher-is-better" |
"Not-applicable", "span": "", "section": "" },
    "outcome_is_primary":   { "value": <bool>, "span": "", "section": "" },
    "outcome_timepoint_weeks": { "value": <num|null>, "span": "", "section":
"" },
    "outcome_timepoint_label": { "value": "Baseline" | "Mid-intervention" | "End-
of-intervention" | "Post-intervention" | "1-month" | "3-month" | "6-month" | "9-
month" | "12-month" | "18-month" | "24-month" | "Study-defined-other", "span": "",
"section": "" },
    "outcome_timepoint_verbatim": { "value": "<verbatim label from source>",
"span": "", "section": "" },
    "outcome_eff_metric":   { "value": "SMD" | "Cohen d" | "Hedges g" | "Mean-
diff" | "Mean-change" | "Adjusted-mean-diff" | "RR" | "OR" | "HR" | "IRR" |
"Remission-rate" | "Response-rate" | "Recovery-rate" | "Correlation" | "Other",

```

```

"span": "", "section": "" },
  "outcome_eff_value": { "value": <num|null>, "span": "", "section": "" },
  "outcome_eff_se": { "value": <num|null>, "span": "", "section": "" },
  "outcome_eff_ci_lo": { "value": <num|null>, "span": "", "section": "" },
  "outcome_eff_ci_hi": { "value": <num|null>, "span": "", "section": "" },
  "outcome_eff_p": { "value": "<free text as reported>", "span": "",
"section": "" },
  "outcome_n_int": { "value": <int|null>, "span": "", "section": "" },
  "outcome_n_ctrl": { "value": <int|null>, "span": "", "section": "" },
  "outcome_arm_int_mean": { "value": <num|null>, "span": "", "section": "" },
  "outcome_arm_int_sd": { "value": <num|null>, "span": "", "section": "" },
  "outcome_arm_ctrl_mean": { "value": <num|null>, "span": "", "section": "" },
  "outcome_arm_ctrl_sd": { "value": <num|null>, "span": "", "section": "" },
  "outcome_eff_direction": { "value": "Favours-intervention" | "Null" |
"Favours-control" | "Unclear", "span": "", "section": "" },
  "outcome_adjusted_flag": { "value": <bool>, "span": "", "section": "" },
  "outcome_adjustment_covariates": { "value": "<free text>", "span": "",
"section": "" },
  "outcome_subgroup": { "value": "<free text>", "span": "", "section": "" },
  "outcome_source_location": { "value": "<free text e.g. 'Table 3, row 4'>",
"span": "", "section": "" },
  "outcome_quote": { "value": "<verbatim ≤ 40 words>", "span": "",
"section": "" }
},
],
"trajectory_shape": { "value": "Sustained" | "Decaying" | "Rebounding" |
"Sleeper" | "Not-reported", "span": "", "section": "" },
"sleeper_effect_flag": { "value": <bool>, "span": "", "section": "" },
"mechanism_claim": { "value": "<free text or null>", "span": "", "section": "" },
"mediation_analysis": { "value": <bool>, "span": "", "section": "" },
"mediator_named": { "value": "<free text or null>", "span": "", "section": "" },

// Engagement (needed for RQ15)
"engage_attendance_pct": { "value": <num|null>, "span": "", "section": "" },
"engage_completion_pct": { "value": <num|null>, "span": "", "section": "" },
"engage_dropout_pct": { "value": <num|null>, "span": "", "section": "" },
"engage_reasons": { "value": "<free text>", "span": "", "section": "" },
"engage_quote": { "value": "<verbatim if any engage_* set>", "span": "",
"section": "" },

// Safety (needed for RQ17)
"ae_reported": { "value": <bool>, "span": "", "section": "" },
"ae_type": { "value": "<free text if ae_reported>", "span": "", "section": "" },
"ae_rate": { "value": "<free text if ae_reported>", "span": "", "section": "" },
"safety_pathway": { "value": <bool>, "span": "", "section": "" },
"safety_pathway_desc": { "value": "<free text if safety_pathway>", "span": "",
"section": "" },
"safety_quote": { "value": "<verbatim if ae_reported or safety_pathway>",
"span": "", "section": "" },
"safety_monitoring_practice": { "value": ["<multi-select>"], "span": "", "section":

```

```

"" },
"referral_pathway_type": { "value": "<single-select>", "span": "", "section":
"" },
"step_up_trigger_ae": { "value": "<free text>", "span": "", "section": "" },

// Cost (needed for RQ16; other RQs opportunistically)
"cost_reported": { "value": <bool>, "span": "", "section": "" },
"cost_per_pt_local": { "value": <num|null>, "span": "", "section": "" },
"cost_currency": { "value": "<ISO-4217 or null>", "span": "", "section": "" },
"cost_year": { "value": <int|null>, "span": "", "section": "" },
"cost_perspective": { "value": "Provider" | "Societal" | "Health-system" | "Mixed" |
"Not-reported", "span": "", "section": "" },
"cost_components": { "value": "<free text>", "span": "", "section": "" },
"cost_metric": { "value": [<multi-select>], "span": "", "section": "" },
"cost_driver": { "value": [<multi-select>], "span": "", "section": "" },
"cost_driver_share_pct": { "value": "<free text>", "span": "", "section": "" },
"cost_effectiveness_ratio": { "value": "<free text or null>", "span": "", "section": ""
},
"ce_threshold_used": { "value": "<free text or null>", "span": "", "section": "" },
"sensitivity_analysis_flag": { "value": <bool>, "span": "", "section": "" },
"cost_quote": { "value": "<verbatim if cost_reported>", "span": "", "section":
"" },

// Instrument validation (RQ18)
"psychom_instrument": { "value": "<D.3 sev_scale>", "span": "", "section": "" },
"psychom_setting": { "value": "<free text>", "span": "", "section": "" },
"psychom_language": { "value": [<free text list>], "span": "", "section": "" },
"psychom_sensitivity": { "value": <num|null>, "span": "", "section": "" },
"psychom_specificity": { "value": <num|null>, "span": "", "section": "" },
"psychom_cutoff": { "value": <num|null>, "span": "", "section": "" },
"psychom_reliability": { "value": "<free text or null>", "span": "", "section": "" },
"psychom_construct": { "value": "<free text or null>", "span": "", "section":
"" },
"psychom_quote": { "value": "<verbatim if any psychom_* set>", "span": "",
"section": "" },

// RQ1 driver-specific
"driver_domain": { "value": ["Biological" | "Psychological" | "Social" |
"Economic" | "Structural" | "Environmental" | "Life-course"], "span": "", "section":
"" },
"driver_named": { "value": [<free text list, verbatim wording>], "span": "",
"section": "" },
"driver_direction": { "value": "<free text>", "span": "", "section": "" },
"driver_effect_size": { "value": "<free text>", "span": "", "section": "" },
"driver_ssa_flag": { "value": <bool>, "span": "", "section": "" },
"driver_quote": { "value": "<verbatim per named driver>", "span": "",
"section": "" },

// RQ2 programme-family
"programme_family": { "value": ["IPT-G" | "PM+" | "HAP" | "Friendship-Bench" |

```



```

"THP" | "BA-based" | "PST-based" | "MI-based" | "SSI-protocolised" | "Mixed" |
"Other"], "span": "", "section": "" },
  "rationale_stated": { "value": "<free text>", "span": "", "section": "" },
  "feasibility_evidence": { "value": "<free text>", "span": "", "section": "" },
  "outcome_timeframe": { "value": ["short" | "medium" | "long"], "span": "",
"section": "" },

// RQ3 scalability
"scalability_parameters": { "value": ["<multi-select>"], "span": "", "section": "" },
"scalability_direction": { "value": "<free text>", "span": "", "section": "" },
"scalability_barriers": { "value": "<free text>", "span": "", "section": "" },
"scalability_enablers": { "value": "<free text>", "span": "", "section": "" },

// RQ4 training/supervision
"train_curriculum": { "value": "<free text>", "span": "", "section": "" },
"train_duration_h": { "value": <num|null>, "span": "", "section": "" },
"train_boosters": { "value": "<free text>", "span": "", "section": "" },
"super_frequency": { "value": "Weekly" | "Fortnightly" | "Monthly" | "Ad-hoc" |
"Not-reported", "span": "", "section": "" },
"super_modality": { "value": "Individual" | "Group" | "Case-based" | "Peer-to-
peer" | "Cascade-with-specialist" | "Cascade-lay-only", "span": "", "section": "" },
"super_specialist_backstop": { "value": <bool>, "span": "", "section": "" },
"fac_selection_criteria": { "value": "<free text>", "span": "", "section": "" },

// RQ5 / RQ6 component
"component_contribution": { "value": "<free text per technique>", "span": "",
"section": "" },
"component_evidence_type": { "value": "Dismantling-MA" | "Component-MA" |
"Network-MA" | "Meta-regression" | "Head-to-head-trials" | "Narrative" | "Other",
"span": "", "section": "" },
"dismantling_flag": { "value": <bool>, "span": "", "section": "" },
"component_drop_candidate": { "value": "<free text>", "span": "", "section": "" },
"component_null_evidence": { "value": "<free text>", "span": "", "section": "" },
"cost_savings_if_dropped": { "value": "<free text>", "span": "", "section": "" },

// RQ7 dose-response
"dose_response_curve": { "value": "Linear" | "Log-linear" | "Plateau" | "U-
shaped" | "Threshold" | "Not-reported", "span": "", "section": "" },
"dose_response_inflection": { "value": "<free text or numeric>", "span": "",
"section": "" },
"dose_response_by_severity": { "value": "<free text>", "span": "", "section": "" },
"specialist_delivered_flag": { "value": <bool>, "span": "", "section": "" },

// RQ9 SSI vs MSI
"ssi_vs_msi_comparison": { "value": "Head-to-head" | "Indirect" | "Not-compared",
"span": "", "section": "" },
"ssi_effect_size": { "value": <num|null>, "span": "", "section": "" },
"msi_effect_size": { "value": <num|null>, "span": "", "section": "" },
"ssi_appropriate_conditions": { "value": "<free text>", "span": "", "section": "" },
"triage_rule_reported": { "value": <bool>, "span": "", "section": "" },

```

```

"triage_threshold":    { "value": "<free text>", "span": "", "section": "" },

// RQ10 group size
"group_size_reported": { "value": "Exact" | "Range" | "Modal" | "Not-reported",
"span": "", "section": "" },
"group_size_effect_evidence": { "value": "<free text>", "span": "", "section": "" },
"group_size_upper_threshold": { "value": "<free text or numeric>", "span": "",
"section": "" },
"fidelity_score_at_size":    { "value": "<free text>", "span": "", "section": "" },

// RQ11 spillover
"sample_type":    { "value": "Universal-prevention" | "Selective-prevention" |
"Indicated" | "Household" | "Community" | "Mixed", "span": "", "section": "" },
"non_case_definition": { "value": "<free text>", "span": "", "section": "" },
"spillover_direction": { "value": "Symptom-reduction" | "Wellbeing-gain" |
"Service-uptake" | "Null" | "Adverse", "span": "", "section": "" },
"spillover_target":    { "value": "<free text>", "span": "", "section": "" },
"spillover_mechanism": { "value": "<free text>", "span": "", "section": "" },

// RQ12 stepped care
"stepped_care_steps": { "value": "<free text describing each step>", "span": "",
"section": "" },
"allocation_rule":    { "value": "Universal" | "Triaged" | "Stratified" | "Adaptive" |
"Not-reported", "span": "", "section": "" },
"step_up_rule":    { "value": "<free text>", "span": "", "section": "" },
"step_down_rule":    { "value": "<free text>", "span": "", "section": "" },
"specialist_backstop": { "value": <bool>, "span": "", "section": "" },

// RQ13 non-specialist delivery
"delivery_model":    { "value": "Lay-only" | "Lay-with-specialist-supervision" |
"Peer-only" | "Peer-with-supervision" | "Self-guided-with-facilitator" | "Mixed",
"span": "", "section": "" },
"hcd_features":    { "value": [<multi-select>], "span": "", "section": "" },
"non_specialist_vs_specialist_delta": { "value": "<free text>", "span": "", "section":
"" },

// RQ14 trajectory
"trajectory_by_model": { "value": "<free text>", "span": "", "section": "" },
"relapse_reported":    { "value": <bool>, "span": "", "section": "" },
"relapse_rate":    { "value": <num|null>, "span": "", "section": "" },
"sustainability_conditions": { "value": "<free text>", "span": "", "section": "" },

// RQ15 engagement design drivers
"engagement_driver": { "value": [<multi-select>], "span": "", "section": "" },
"engagement_barrier": { "value": [<multi-select>], "span": "", "section": "" },
"adherence_definition": { "value": "<free text>", "span": "", "section": "" },
"engagement_effect_size": { "value": "<free text>", "span": "", "section": "" },

// RQ17 safety adequacy

```

```

"adequacy_evidence": { "value": "<free text>", "span": "", "section": "" },
"adequacy_gap":      { "value": "<free text>", "span": "", "section": "" },

//
=====
=====
// [REVISION — pending user review]
// Primary-study-specific fields (populate ONLY when
// unit_of_extraction = "primary_study"; set null with span "" and do
// NOT flag for human when unit_of_extraction = "review")
//
=====
=====

// 3A.4.1 Sourcing and tier
"record_tier":      { "value": "full" | "reduced", "span": "", "section": "" },
"source_review_ids": { "value": ["<review sid>"], "span": "", "section": "" },
"linked_publications": { "value": ["<DOI or citation of companion pub>"], "span":
"", "section": "" },

// 3A.4.2 RCT quality items (feed fatal-flaw ROB)
"rct_randomisation_method": { "value": "<free text>", "span": "", "section": "" },
"rct_allocation_concealment": { "value": "Y" | "N" | "Unclear" | "NA", "span": "",
"section": "" },
"rct_blinding_participants": { "value": "Y" | "N" | "Unclear" | "NA", "span": "",
"section": "" },
"rct_blinding_outcome_assessor": { "value": "Y" | "N" | "Unclear" | "NA", "span":
"", "section": "" },
"rct_itt_analysis":      { "value": "Y" | "N" | "Unclear" | "NA", "span": "",
"section": "" },
"rct_cluster_unit":      { "value": "<free text>", "span": "", "section": "" },
"rct_icc_reported":      { "value": <bool>, "span": "", "section": "" },
"rct_icc_value":         { "value": <num|null>, "span": "", "section": "" },
"quasi_confounder_adjustment": { "value": "<free text>", "span": "", "section":
"" },
"rob_quote":            { "value": "<verbatim>", "span": "", "section": "" },

// 3A.4.6 Fatal-Flaw ROB checklist [REVISION — pending user review]
// Meta
"ffrob_coding_pair":      { "value": "<free text>", "span": "", "section": "" },
"ffrob_minutes":         { "value": <num|null>, "span": "", "section": "" },
"ffrob_reviewer_notes":   { "value": "<free text>", "span": "", "section": "" },

// Criterion 1 — Confounding / baseline non-equivalence
"ffrob_c1a":             { "value": "No — proceed" | "Yes — STOP, high ROB",
"span": "", "section": "" },
"ffrob_c1a_span":        { "value": "<verbatim if c1a=Yes>", "span": "",
"section": "" },
"ffrob_c1b":             { "value": "No — proceed" | "Yes — STOP, high ROB",
"span": "", "section": "" },

```

```

"ffrob_c1b_span":      { "value": "<verbatim if c1b=Yes>", "span": "",
"section": "" },
"ffrob_c1_notes":      { "value": "<free text if c1 fails>", "span": "", "section":
"" },

```

// Criterion 2 — Differential attrition

```

"ffrob_c2a":           { "value": "No — proceed" | "Yes — STOP, high ROB",
"span": "", "section": "" },
"ffrob_c2a_span":      { "value": "<verbatim if c2a=Yes>", "span": "",
"section": "" },
"ffrob_c2b":           { "value": "No — proceed" | "Yes — STOP, high ROB",
"span": "", "section": "" },
"ffrob_c2b_span":      { "value": "<verbatim if c2b=Yes>", "span": "",
"section": "" },
"ffrob_c2_notes":      { "value": "<free text if c2 fails>", "span": "", "section":
"" },

```

// Criterion 3 — Outcome measurement influenced by treatment

```

"ffrob_c3a":           { "value": "No — proceed" | "Yes — STOP, high ROB",
"span": "", "section": "" },
"ffrob_c3a_span":      { "value": "<verbatim if c3a=Yes>", "span": "",
"section": "" },
"ffrob_c3b":           { "value": "No — proceed" | "Yes — STOP, high ROB",
"span": "", "section": "" },
"ffrob_c3b_span":      { "value": "<verbatim if c3b=Yes>", "span": "",
"section": "" },
"ffrob_c3_notes":      { "value": "<free text if c3 fails>", "span": "", "section":
"" },

```

// Criterion 4 — Selective reporting (methods vs results only)

```

"ffrob_c4":           { "value": "No — proceed" | "Yes — STOP, high ROB",
"span": "", "section": "" },
"ffrob_c4_span":      { "value": "<verbatim (quote methods and omitted
results)>", "span": "", "section": "" },
"ffrob_c4_notes":      { "value": "<free text if c4 fails>", "span": "", "section":
"" },

```

// Criterion 5 — Contamination / intervention deviations

```

"ffrob_c5a":           { "value": "No — proceed" | "Yes — STOP, high ROB",
"span": "", "section": "" },
"ffrob_c5a_span":      { "value": "<verbatim if c5a=Yes>", "span": "",
"section": "" },
"ffrob_c5b":           { "value": "No — proceed" | "Yes — STOP, high ROB",
"span": "", "section": "" },
"ffrob_c5b_span":      { "value": "<verbatim if c5b=Yes>", "span": "",
"section": "" },
"ffrob_c5_notes":      { "value": "<free text if c5 fails>", "span": "", "section":
"" },

```

// Overall

```
"ffrob_overall_decision":    { "value": "Assign for full ROB assessment" | "High
ROB — fatal flaw", "span": "", "section": "" },
"ffrob_failed_criteria":    { "value": ["C1a" | "C1b" | "C2a" | "C2b" | "C3a" | "C3b"
| "C4" | "C5a" | "C5b"], "span": "", "section": "" },
```

```
// 3A.4.3 Attrition
"attrition_overall_pct":    { "value": <num|null>, "span": "", "section": "" },
"attrition_int_pct":       { "value": <num|null>, "span": "", "section": "" },
"attrition_ctrl_pct":      { "value": <num|null>, "span": "", "section": "" },
"differential_attrition_flag": { "value": <bool>, "span": "", "section": "" },
"baseline_balance":        { "value": "<free text>", "span": "", "section": "" },
"attrition_quote":         { "value": "<verbatim if any attrition_* set>", "span": "",
"section": "" },
```

```
// 3A.4.4 Trial provenance
"trial_registered":        { "value": <bool>, "span": "", "section": "" },
"trial_registry_id":       { "value": "<free text>", "span": "", "section": "" },
"protocol_published":      { "value": <bool>, "span": "", "section": "" },
"primary_outcome_prespec":  { "value": "Y" | "N" | "Unclear", "span": "",
"section": "" },
"funding_source":          { "value": "<free text>", "span": "", "section": "" },
"coi_declared":            { "value": <bool>, "span": "", "section": "" },
"coi_description":         { "value": "<free text>", "span": "", "section": "" },
"provenance_quote":        { "value": "<verbatim>", "span": "", "section": "" },
```

```
// 3A.4.5 Analytic sample by arm
"n_enrolled_int":          { "value": <int|null>, "span": "", "section": "" },
"n_enrolled_ctrl":         { "value": <int|null>, "span": "", "section": "" },
"n_randomised_int":        { "value": <int|null>, "span": "", "section": "" },
"n_randomised_ctrl":       { "value": <int|null>, "span": "", "section": "" },
"arm_definitions":         { "value": "<free text>", "span": "", "section": "" },
"arm_quote":               { "value": "<verbatim if any arm-level N populated>",
"span": "", "section": "" },
```

```
//
=====
=====
// END REVISION block
//
=====
=====
```

```
//
=====
=====
// [REVISION — pending user review]
// AMSTAR-2 itemised checklist (review-level records only)
// Set every amstar2_i{N} field to null with empty span when
// unit_of_extraction = "primary_study" — do NOT flag for human.
//
=====
```

=====

```
"amstar2_mode":      { "value": "Full (16-item)" | "Rapid critical-domains (7-
item)", "span": "", "section": "" },

// Item 1 (non-critical)
"amstar2_i1":        { "value": "Yes" | "No", "span": "", "section": "" },
"amstar2_i1_span":    { "value": "<verbatim>", "span": "", "section": "" },
"amstar2_i1_notes":   { "value": "<free text>", "span": "", "section": "" },

// Item 2 (CRITICAL)
"amstar2_i2":         { "value": "Yes" | "Partial Yes" | "No", "span": "", "section":
"" },
"amstar2_i2_span":    { "value": "<verbatim>", "span": "", "section": "" },
"amstar2_i2_notes":   { "value": "<free text>", "span": "", "section": "" },

// Item 3 (non-critical)
"amstar2_i3":         { "value": "Yes" | "No", "span": "", "section": "" },
"amstar2_i3_span":    { "value": "<verbatim>", "span": "", "section": "" },
"amstar2_i3_notes":   { "value": "<free text>", "span": "", "section": "" },

// Item 4 (CRITICAL)
"amstar2_i4":         { "value": "Yes" | "Partial Yes" | "No", "span": "", "section":
"" },
"amstar2_i4_span":    { "value": "<verbatim>", "span": "", "section": "" },
"amstar2_i4_notes":   { "value": "<free text>", "span": "", "section": "" },

// Item 5 (non-critical)
"amstar2_i5":         { "value": "Yes" | "No", "span": "", "section": "" },
"amstar2_i5_span":    { "value": "<verbatim>", "span": "", "section": "" },
"amstar2_i5_notes":   { "value": "<free text>", "span": "", "section": "" },

// Item 6 (non-critical)
"amstar2_i6":         { "value": "Yes" | "No", "span": "", "section": "" },
"amstar2_i6_span":    { "value": "<verbatim>", "span": "", "section": "" },
"amstar2_i6_notes":   { "value": "<free text>", "span": "", "section": "" },

// Item 7 (CRITICAL)
"amstar2_i7":         { "value": "Yes" | "Partial Yes" | "No", "span": "", "section":
"" },
"amstar2_i7_span":    { "value": "<verbatim>", "span": "", "section": "" },
"amstar2_i7_notes":   { "value": "<free text>", "span": "", "section": "" },

// Item 8 (non-critical)
"amstar2_i8":         { "value": "Yes" | "Partial Yes" | "No", "span": "", "section":
"" },
"amstar2_i8_span":    { "value": "<verbatim>", "span": "", "section": "" },
"amstar2_i8_notes":   { "value": "<free text>", "span": "", "section": "" },
```

```

// Item 9 (CRITICAL)
"amstar2_i9":      { "value": "Yes" | "Partial Yes" | "No" | "Includes only NRSI" |
"Includes only RCTs", "span": "", "section": "" },
"amstar2_i9_span":  { "value": "<verbatim>", "span": "", "section": "" },
"amstar2_i9_notes": { "value": "<free text>", "span": "", "section": "" },

// Item 10 (non-critical)
"amstar2_i10":      { "value": "Yes" | "No", "span": "", "section": "" },
"amstar2_i10_span": { "value": "<verbatim>", "span": "", "section": "" },
"amstar2_i10_notes": { "value": "<free text>", "span": "", "section": "" },

// Item 11 (CRITICAL)
"amstar2_i11":      { "value": "Yes" | "No" | "No MA conducted", "span": "",
"section": "" },
"amstar2_i11_span": { "value": "<verbatim>", "span": "", "section": "" },
"amstar2_i11_notes": { "value": "<free text>", "span": "", "section": "" },

// Item 12 (non-critical)
"amstar2_i12":      { "value": "Yes" | "No" | "No MA conducted", "span": "",
"section": "" },
"amstar2_i12_span": { "value": "<verbatim>", "span": "", "section": "" },
"amstar2_i12_notes": { "value": "<free text>", "span": "", "section": "" },

// Item 13 (CRITICAL)
"amstar2_i13":      { "value": "Yes" | "No", "span": "", "section": "" },
"amstar2_i13_span": { "value": "<verbatim>", "span": "", "section": "" },
"amstar2_i13_notes": { "value": "<free text>", "span": "", "section": "" },

// Item 14 (non-critical)
"amstar2_i14":      { "value": "Yes" | "No", "span": "", "section": "" },
"amstar2_i14_span": { "value": "<verbatim>", "span": "", "section": "" },
"amstar2_i14_notes": { "value": "<free text>", "span": "", "section": "" },

// Item 15 (CRITICAL)
"amstar2_i15":      { "value": "Yes" | "No" | "No MA conducted", "span": "",
"section": "" },
"amstar2_i15_span": { "value": "<verbatim>", "span": "", "section": "" },
"amstar2_i15_notes": { "value": "<free text>", "span": "", "section": "" },

// Item 16 (non-critical)
"amstar2_i16":      { "value": "Yes" | "No", "span": "", "section": "" },
"amstar2_i16_span": { "value": "<verbatim>", "span": "", "section": "" },
"amstar2_i16_notes": { "value": "<free text>", "span": "", "section": "" },

// Derived and provenance
"amstar2_critical_weaknesses": { "value": ["I2" | "I4" | "I7" | "I9" | "I11" |
"I13" | "I15"], "span": "", "section": "" },
"amstar2_noncritical_weaknesses_count": { "value": <int|null>, "span": "",
"section": "" },

```

```

    "amstar2_band_derived":          { "value": "High" | "Moderate" | "Low" |
"Critically-low", "span": "", "section": "" },
    "amstar2_override_reason":      { "value": "<free text or null>", "span": "",
"section": "" },
    "amstar2_inherited_source":     { "value": "<sid of umbrella or null>", "span":
"", "section": "" },
    //
=====
=====
    // END AMSTAR-2 revision block
    //
=====
=====

    // Quality / provenance (D.9) — existing summary fields (retained)
    "amstar2_band":                { "value": "High" | "Moderate" | "Low" | "Critically-low",
"span": "", "section": "" },
    "amstar2_inherited":           { "value": <bool>, "span": "", "section": "" },
    "rob_fatal_flaw":              { "value": "Fatal-flaw-present" | "No-fatal-flaw" | "Not-
applicable", "span": "", "section": "" },

    // RQ tagging + report mapping (§3.12)
    "rq_tags":                     { "value": ["RQ1" | "RQ2" | ... | "RQ18"], "span": "", "section":
"" },
    "workstream":                  { "value": ["A" | "B" | "C" | "D"], "span": "", "section": "" },
    "template_section":            { "value": ["<see Section E>"], "span": "", "section": "" },
    "rq11_only":                   { "value": <bool>, "span": "", "section": "" },

    // Study-to-RQ contribution matrix — repeating array (§3.12A) [REVISION]
    // One entry per RQ this study contributes to. Every rq_id here must
    // also appear in rq_tags above. The exported extraction sheet
    // materialises this array as long-format (one row per {sid × rq_id}).
    "rq_contributions": [
    {
        "rq_id":                    { "value": "RQ1" | "RQ2" | ... | "RQ18", "span": "",
"section": "" },
        "rq_contribution_type":      { "value": "Primary-evidence" | "Supporting-
evidence" | "Contextual-evidence" | "Contradictory-evidence" | "Descriptive-only" |
"Not-applicable", "span": "", "section": "" },
        "rq_contribution_summary":   { "value": "<≤ 30 words>", "span": "",
"section": "" },
        "rq_contribution_strength":  { "value": "High" | "Moderate" | "Low" |
"Insufficient", "span": "", "section": "" },
        "rq_contribution_direction": { "value": "Confirms" | "Refutes" | "Qualifies" |
"Neutral" | "Not-applicable", "span": "", "section": "" },
        "rq_contribution_data_fields": { "value": ["<field_id>", "..."], "span": "",
"section": "" },
        "rq_contribution_template_section": { "value": ["<see Section E>"], "span": "",
"section": "" },
        "rq_contribution_quote":     { "value": "<verbatim ≤ 40 words>", "span": "",

```



```

"section": "" }
}
],

// Extractor bookkeeping
"extractor_confidence": <float 0.0-1.0>,
"fields_flagged_for_human": ["<field_id>", ...]
}

```

---

#### D. Controlled vocabularies (closed lists; do not invent values)

---

design (review):

Systematic-review | Meta-analysis | Meta-regression | IPD-meta-analysis  
 | Network-meta-analysis | Umbrella-review | Dismantling-component-meta-analysis  
 | Systematic-review-of-economic-evaluations | Scoping-review | Other-review

design (primary study):

RCT-individual | RCT-cluster | Quasi-experimental-controlled

sev\_strat:

Minimal | Mild | Moderate | Moderately-severe | Severe  
 | MDD-diagnosed-unspecified | Mixed-unstratified  
 | Perinatal-elevated-symptoms | Not-reported

sev\_scale:

PHQ-9 | PHQ-2 | CES-D | CIDI | SCID | MINI | HAMD | BDI | EPDS  
 | SRQ-20 | K10 | Other

int\_techniques (multi-select  $\geq 1$ ):

Psychoeducation | Behavioural-activation | Problem-solving  
 | Cognitive-restructuring | Interpersonal-techniques  
 | Mindfulness-acceptance | Relaxation-stress-management  
 | Behavioural-skills-training | Motivational-interviewing  
 | Supportive-counselling | Narrative-expressive | Peer-support-mutual-aid  
 | Single-session-protocolised | Self-help-bibliotherapy | Other-technique

int\_adaptations (multi-select, optional):

Religious-spiritual-content | Cultural-adaptation-named  
 | Livelihoods-economic-component | Family-or-spouse-engagement-component  
 | Trauma-or-violence-specific-content | HIV-specific-content  
 | Other-adaptation

dose\_band:

Ultra-brief | Brief | Standard-short | Standard | Extended | Long  
 | Stepped-variable | Not-reported

fac\_type:

Lay-paid | Lay-volunteer | Peer | Teacher-educator | Religious-faith-leader  
| Mixed-team | Specialist | Not-reported

del\_format:

Group-face-to-face | Family-couple | Group-remote-phone-or-video  
| Hybrid | Self-help-print-guided

setting:

Primary-care-clinic | Community | School | Workplace  
| Antenatal-postnatal-service | NGO-program | Refugee-humanitarian  
| Digital-only | Home-based

amstar2\_band:

High | Moderate | Low | Critically-low

---

#### E. RQ → template\_section mapping (populate template\_section from this table)

---

For every RQ this record contributes to (rq\_tags), add every listed Decision Brief section to template\_section (multi-select). Sections use the notation "\$X.Y" from the Strongminds template.

RQ1 → §3.1; §6-A-RQ1; §2-dashboard; §7-theme5  
RQ2 → §3.4; §6-A-RQ2; §2-dashboard; §8.4  
RQ3 → §6-C-RQ3; §8.1; §2-dashboard; §7-theme1  
RQ4 → §6-C-RQ4; §8.1; §3.2; §2-dashboard; §7-theme1  
RQ5 → §6-B-RQ5; §8.4; §8.5; §8.6; §9a-Action1; §2-dashboard  
RQ6 → §6-B-RQ6; §8.4; §8.5; §9a-Action1; §2-dashboard  
RQ7 → §6-C-RQ7; §8.2; §8.1; §8.5; §8.6; §9a-Action1; §2-dashboard  
RQ8 → §6-B-RQ8; §8.5; §7-theme5; §9b-Priority1; §10-Gap1; §2-dashboard  
RQ9 → §6-C-RQ9; §8.2; §8.3; §9a-Action1; §9a-Action3; §2-dashboard  
RQ10 → §6-C-RQ10; §8.1; §7-theme3; §9b-Priority4; §2-dashboard  
RQ11 → §6-B-RQ11; §10-Gap4; §2-dashboard  
RQ12 → §6-C-RQ12; §8.3; §8.5; §8.6; §9a-Action3; §2-dashboard  
RQ13 → §6-C-RQ13; §8.1; §8.5; §8.6; §9a-Action2; §2-dashboard  
RQ14 → §6-B-RQ14; §8.4; §8.5; §7-theme5; §9b-Priority1; §2-dashboard  
RQ15 → §6-C-RQ15; §7-theme3; §9b-Priority5; §2-dashboard  
RQ16 → §6-D-RQ16; §8.1; §8.4; §8.5; §8.6; §9a-Action5; §9b-Priority2; §10-Gap2; §2-dashboard  
RQ17 → §6-C-RQ17; §8.3; §7-theme4; §9a-Action4; §9b-Priority3; §10-Gap3; §2-dashboard  
RQ18 → §6-C-RQ18; §8.3; §9a-Action3; §2-dashboard

---

#### F. Document to extract

---

{{segmented\_full\_text}}

---

Return the single JSON object described in Section C. Return JSON only.  
No prose, no markdown, no code fences.

---

### Paraphrase variants (k = 3)

Run the record through all three variants. Take a per-field majority vote. Any per-field disagreement → add the field to `fields_flagged_for_human` in the merged output.

#### Variant EX-1 (base)

The USER block above, unchanged.

#### Variant EX-2 (anti-anchoring; randomised field order)

Reorder the schema in Section C so the fields appear in a shuffled sequence (fix the shuffle per run for reproducibility; store the permutation in `/llm_logs/`). System block and vocabularies unchanged. This tests whether the extractor is anchoring on field order.

#### Variant EX-3 (extract-or-justify)

Add the following sentence at the top of the USER block:

For every field in the schema, either populate it with an extracted value (and its verbatim span) OR set value = null and, in a separate `absence_notes` map keyed by `field_id`, record in ≤ 15 words the reason the field is absent from this document (e.g., “review reports pooled effect only, no per-severity subgroup”). This is a per-field justify-your-absence framing.

Add to the schema:

```
"absence_notes": { "<field_id>": "<≤ 15 words>" }
```

Variant EX-3's `absence_notes` are used only to enrich `fields_flagged_for_human` at the merge step; they are not carried into the final extraction record.

---

### Merge logic (across the k = 3 runs)

For each field: 1. If all three values agree → take the value; span = span from EX-1. 2. If two of three agree → take the majority value; span from a run in the majority; add the field ID to `fields_flagged_for_human`. 3. If all three disagree → set value = null; add field ID to `fields_flagged_for_human`; store all three candidates in `/llm_logs/`. 4. extractor\_confidence in the merged record = mean of the three per-run confidences.

---

## Span-validation call (post-extraction, per protocol Appendix F.5)

For every non-null field in the merged record, call the span-validator prompt with {field\_id, value, span, section} plus the segmented document. Any verdict: "fail" → human re-extraction of that field.

The span-validator runs over 100% of quantitative fields and a 20% random sample of qualitative fields.

Span-validator prompt (unchanged from protocol Appendix F.5):

SYSTEM

You are a verification function. You receive (a) a claimed value, (b) a verbatim span, (c) the section it was drawn from, and (d) the full segmented document. You verify that the span appears in the document EXACTLY as quoted (character-for-character, including punctuation and digits) AND that the section reference is accurate AND that the claimed value is consistent with what the span says. You return JSON only.

USER

Verify the following claim.

claim:

```
field: "{{field_id}}"
value: {{value}}
span: "{{span}}"
section: "{{section}}"
```

document:

```
{{segmented_full_text}}
```

Return:

```
{
  "span_present_verbatim": true | false,
  "section_correct": true | false,
  "value_consistent_with_span": true | false,
  "verdict": "pass" | "fail",
  "failure_reason": "<≤25 words; empty if pass>"
}
```

verdict = "pass" iff all three booleans are true.

---

## Calibration before live use

Before running this prompt on the full corpus, calibrate against the human gold standard (5 pilot studies per protocol §4.4). Iterate the prompt (log every change in the deviation register) until per-field agreement with the human extraction reaches an acceptable level. Only then deploy.

Every prompt version, model API string, seed, and full response is stored as JSONL under /llm\_logs/.

---

## Change log

| Version | Date    | Change                                                                                                                                                                                                                                                                                                                |
|---------|---------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 1.0     | 2026-07 | Initial prompt derived from Protocol Appendix F.4, expanded with RQ-specific fields per ULCM_M1_Extraction_Fields_by_RQ.docx.                                                                                                                                                                                         |
| 1.1     | 2026-07 | Added eff_se, arm-level Ns (eff_n_int, eff_n_ctrl), and per-time-point SE / CI / arm-Ns to the durability block, per synthesis requirement for weighted effect-size tabulation.                                                                                                                                       |
| 1.2     | 2026-07 | <b>[REVISION — pending user review]</b> Added primary-study extraction workflow (\$ new rules 11–16), record-tier flag (full vs reduced), sourcing-review link (source_review_ids), RCT quality items (\$3A.4.2), attrition (\$3A.4.3), trial provenance (\$3A.4.4), and arm-level enrolled/randomised Ns (\$3A.4.5). |
| 1.3     | 2026-07 | <b>[REVISION — pending user review]</b> Added Fatal-Flaw ROB checklist (\$3A.4.6): 5 criteria × 9 sub-items × Yes/No verdict + per-criterion notes + verbatim spans. Propagates to rob_fatal_flaw. Rules 17–19. Sourced from FF-ROB checklist template shared by the review team.                                     |
| 1.4     | 2026-07 | <b>[REVISION — pending</b>                                                                                                                                                                                                                                                                                            |

|     |         |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        |
|-----|---------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|     |         | <p><b>user review]</b> Added AMSTAR-2 itemised checklist (§3.11A): 16 items × verdict + span + notes, seven critical (I2, I4, I7, I9, I11, I13, I15) + nine non-critical, with rapid critical-domains mode, confidence-band derivation from Shea 2017 rules, and explicit tie-in to the review-only RQs (RQ1–4, 11, 15, 17, 18) where AMSTAR-2 is the sole quality signal. Rules 20–26.</p>                                                                                                                                                                                                                                                            |
| 1.5 | 2026-07 | <p><b>[REVISION — pending user review]</b> Added: (a) §3.4A sub-module decomposition — 11 named programmes with sub-module vocabularies, session-share %, contribution, and evidence type (rule 27); (b) §3.8A repeating additional_outcomes array — every {construct × instrument × timepoint × subgroup} with full effect / arm-mean / SD / N / adjustment fields (rule 28); (c) §3.12A rq_contributions array — one entry per {study × RQ} with contribution type, strength, direction, backing fields, and Decision Brief mapping (rule 29). Exported extraction sheet materialises rq_contributions as long-format one-row-per-{sid × rq_id}.</p> |
| 1.6 | 2026-07 | <p>Constrained §3.8A to DEPRESSION OUTCOMES ONLY per review-team</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |

decision.  
outcome\_construct  
restricted to {Depression-  
symptoms, Depression-  
remission, Depression-  
response, Depression-  
recovery, Depression-  
relapse}. outcome\_scale  
restricted to validated  
depression instruments  
(PHQ-9, PHQ-2, CES-D,  
HAMD/HDRS, MADRS,  
BDI/BDI-II, EPDS, QIDS,  
Zung-SDS, GDS, DASS-21-  
dep, SCL-90-dep, SRQ-20-  
dep, K10-dep-cutoff, MINI-  
dep, CIDI-dep, SCID-dep,  
Other-depression-  
instrument). Non-  
depression outcomes stay  
in existing dedicated blocks  
(\$3.7 cost, \$3.9  
engage/safety, \$3.10  
psychometrics, \$3A.4.3  
attrition).